AMERICAN PSYCHOLOGICAL ASSOCIATION

Psychological Review

ISSN: 0033-295X

2025, Vol. 132, No. 6, 1467–1492
https://doi.org/10.1037/rev0000589

Common Factors in Altered States: Understanding Psychedelic Therapy Through the Lens of Grawe's General Change Mechanisms

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Psychedelic therapy is commonly understood as a form of psychotherapy. However, despite promising results from clinical trials, scientific progress has been hindered by a lack of theoretical integration between the fields of psychedelic and psychotherapy research. This article seeks to bridge this divide by outlining a transtheoretical understanding of psychedelic-occasioned psychological change based on Klaus Grawe's model of *general change mechanisms*—an empirically grounded framework of psychotherapeutic "common factors" that emphasizes therapeutic experiences and change processes. Drawing on extensive evidence from psychedelic research, we argue that the efficacy of psychedelic therapy is ultimately mediated by the same five general change mechanisms that underlie all effective psychotherapies: (1) resource activation, (2) therapeutic relationship, (3) problem actuation, (4) clarification, and (5) mastery. Implications for clinical practice, training, and future research are discussed.

Keywords: psychedelic therapy, psychotherapy, general change mechanisms, common factors, therapy process

> Doing research-informed psychotherapy means to be open for the whole spectrum of therapeutic possibilities, taking elements that already exist and have proven themselves and fashioning them into even more effective procedures. This should not be done in an eclectic manner, but rather on the basis of a new coherent model whose scope is broad enough to encompass the wide variety of therapeutic possibilities. (Grawe, 1997, p. 17)

Psychedelic therapy, defined as the clinical use of psilocybin, lysergic acid diethylamide (LSD), or other psychedelics to promote psychological change, is undergoing a surge in psychiatric research. In recent years, clinical trials have shown promising results for various mental health conditions, including depression, anxiety disorders, substance use disorders, and distress associated with chronic or

This article was published Online First October 9, 2025.
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This work was supported by Bundesministerium für Bildung und Forschung (Grant 01EN2006A), awarded to Gerhard Gründer.

Max Wolff played a lead role in conceptualization and writing–original draft. Hans Rutrecht played an equal role in writing–review and editing. Lea J. Mertens played an equal role in writing–review and editing. Andy A. Meijer played an equal role in writing–review and editing. Jessica Beck played an equal role in writing–review and editing. Sergio Pérez played an equal role in writing–review and editing. Gerhard Gründer played an equal role in writing–review and editing. Isabel Dziobek played a lead role in writing–review and editing. Henrik Jungaberle played a supporting role in conceptualization and an equal role in writing–review and editing.

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terminal illness (Mitchell & Anderson, 2024). Unlike other psychiatric pharmacotherapies, psychedelic therapy is characterized by a close interweaving of pharmacological and psychotherapeutic elements. Psychedelics have been described as "catalysts" of psychotherapeutic processes (Garcia-Romeu & Richards, 2018) and even as "psychotherapeutic drugs" (Müller et al., 2020). Throughout the medical history of these substances, many researchers have agreed that *psychedelic therapy is psychotherapy* (Gründer et al., 2024; Nayak & Johnson, 2021; Nichols & Walter, 2021). However, to date, psychedelic research and psychotherapy research remain separate fields that hardly interact. This lack of cross-disciplinary exchange risks that efforts are invested in reinventing the wheel rather than building upon established knowledge that diverse forms of psychotherapy achieve psychological change in similar ways (Castonguay et al., 2015). To provide common ground for both fields, the present article outlines a theoretical understanding of psychedelic-occasioned psychological change that draws on concepts established by empirical psychotherapy research. We begin with a summary of how psychedelic therapy is implemented in modern clinical trials, followed by a brief introduction to psychedelic drug effects. We then turn to psychotherapy research, introducing Grawe's (1997, 2004) *consistency theory* and the related concept of *general change mechanisms* of psychotherapy. Finally, we draw empirical and theoretical connections between these models and psychedelic research, arguing that psychedelic therapy ultimately operates on the same principles of change that are shared by all effective forms of psychotherapy.

Implementation of Psychedelic Therapy in Modern Clinical Studies

Psychedelic therapies conducted in modern clinical trials follow a broadly circumscribed standard protocol (for a summary, see Garcia-Romeu & Richards, 2018) where a psychedelic substance is administered between one and three times within a psychotherapeutic framework. Patients are commonly accompanied by two therapists throughout all therapy sessions. *Preparatory sessions* conducted at the beginning of the treatment and before each psychedelic dose serve the purpose of building rapport and preparing the patient for the experience of psychedelic states. *Dosing sessions* take place in a living-room-like setting designed to feel comfortable and safe. Depending on the substance used, these sessions can last several hours (psilocybin: ~6 hr; LSD: 8–12 hr), during which the patient is always attended and, if necessary, supported by therapists. During the acute drug effect, verbal exchange is usually kept minimal, as the patient is encouraged to engage in introspection while resting on a bed or sofa, often wearing eyeshades and headphones. Carefully selected music is widely regarded as an important supportive element of this treatment and is included in most clinical protocols, where it is assumed to facilitate processes such as emotional engagement, meaning-making, and mental imagery (Barrett et al., 2018; but see Gloeckler et al., 2025). In the following days and weeks, *integration sessions* are conducted to make therapeutic use of the patient's psychedelic experience and help bring about lasting psychological change.

Since classical psychedelics are considered to have low addictive potential and low toxicity, their harm potential is regarded as relatively small compared to other psychoactive drugs (Johnson et al., 2019). However, all psychedelic drugs pose risks for human abuse (Heal et al., 2018), and human research with psychedelics requires dedicated efforts to minimize potential harms (Johnson et al., 2008). Standard safety measures involve the exclusion of individuals who may be susceptible to psychedelic-induced harm, such as those with a family history of psychosis. Furthermore, the supportive setting serves to protect against overwhelming distress that might entail psychological harm or dangerous behaviors. Severe adverse events are rare in clinical psychedelic research but can occur even with the most careful approaches (Bradberry et al., 2022). "Flashbacks" reported in controlled studies (i.e., brief reoccurrences of drug-like experiences after acute effects have subsided) can be associated with distress, although they are typically mild, short-lived, and perceived as neutral or pleasant (Müller et al., 2022). Hallucinogen persisting perception disorder (Martinotti et al., 2018) has not yet been observed in clinical trials but still requires monitoring.

A Contextual-Behavioral Model of Psychedelic Drug Effects

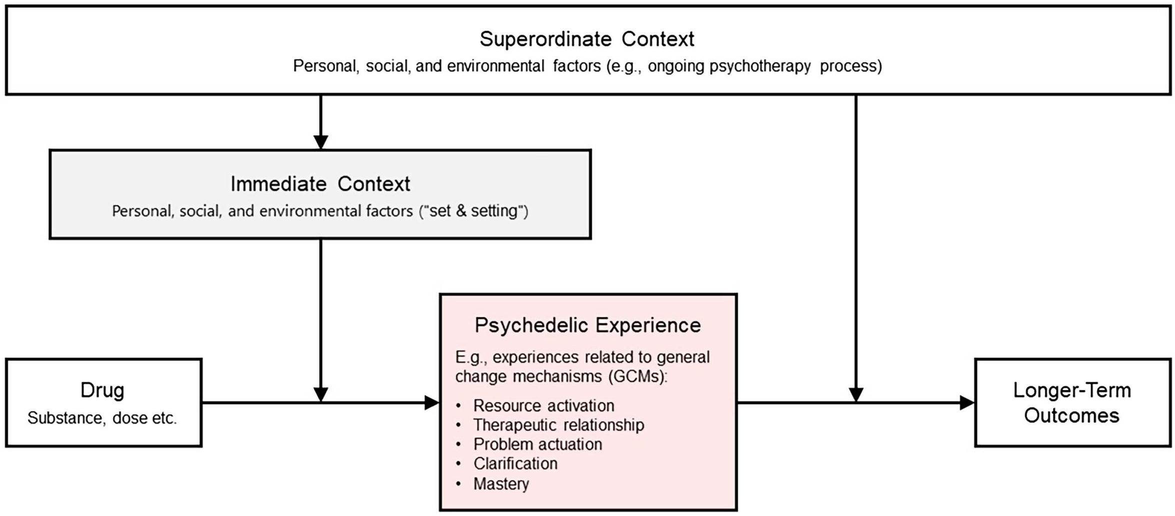
The acute subjective effect of classical psychedelic drugs, or *psychedelic experience*, is characterized as a phenomenologically variable and highly unstable altered state of consciousness that involves complex changes in perception, cognition, and affect. Salient themes include dreamlike experiences (often related to biographical events), increased emotionality, insightfulness, and meaningfulness. Alterations in the sense of self, ranging from subtle (described as a "softening of ego") to drastic (complete "ego-dissolution"), are commonly reported (for summaries of acute subjective effects, see Garcia-Romeu & Richards, 2018; Swanson, 2018).

The contextual–experiential model depicted in Figure 1 (Wolff et al., 2024) combines two core assumptions about psychedelics' effects that form a conceptual framework for the arguments advanced in this article. The first assumption (represented by the horizontal arrows) is that longer term effects, such as sustained symptom reductions or other changes in a person's experience and behavior, are substantially mediated by the acute psychedelic experience. That psychometrically measurable qualities of the psychedelic experience (discussed in detail below) can predict longer term outcomes is indeed one of the best established results of psychedelic research (Aday et al., 2020; Romeo et al., 2025; but see Olson, 2020). This is compatible with the view that certain types of psychedelic experiences can be understood as psychotherapeutic learning experiences (Wolff et al., 2024). Importantly, relevant processes such as reward and extinction learning or semantic memory formation can be modulated in acute psychedelic states, which may contribute to both therapeutic effects and adverse outcomes (Doss, DeMarco, et al., 2024; Kanen et al., 2023; McGovern et al., 2024).

The second assumption (represented by the vertical arrows in Figure 1) is that both the acute psychedelic experience and its longer term effects are strongly influenced by nonpharmacological contextual factors. Substantial context-dependence is widely regarded as a core characteristic of acute psychedelic drug effects, has been well documented in observational research, and is increasingly supported by experimental findings (Aday et al., 2021; Carhart-Harris, Roseman, et al., 2018; Stoliker, Novelli, et al., 2025). Less empirically studied, but nonetheless widely assumed as the rationale for conducting postdosing integration sessions, is the view that contextual factors can also exert a decisive influence on the learning

Figure 1

Contextual–Experiential Model Illustrating the Assumed Mediation of Longer Term Outcomes by the Psychedelic Experience



Note. Immediate and superordinate context factors are assumed to exert moderating effects on the acute drug effect and the learning processes (commonly referred to as "integration") that mediate between the psychedelic experience and longer term outcomes. See the online article for the color version of this figure.

processes that mediate between the acute psychedelic experience and longer term psychological changes (broadly referred to as "integration"; Bathje et al., 2022; Garcia-Romeu & Richards, 2018).

To better differentiate between these "early" and "late" contextual influences than other extrapharmacological models of psychedelic drug effects (e.g., Carhart-Harris, Roseman, et al., 2018), we propose a distinction between immediate and superordinate context factors. This distinction is conceptually related to the difference between states and traits in psychology or between proximal and distal influences in psychotherapy research. The *immediate context* (or "set and setting") refers to the situational conditions of a psychedelic experience. Immediate internal context factors ("set") of particular psychotherapeutic importance may include the expectations or intentions that are present during the experience, motor behavior (e.g., lying down or actively interacting with the environment), attention (e.g., introspective or externally focused), and attitudes toward unexpected or unpleasant experiences (e.g., open/accepting or defensive/avoidant). Relevant external immediate context factors ("setting") may include the physical environment (e.g., the furnishings and lighting of the therapy room; the music being played), in-session events (e.g., disruptions), and the presence and behavior of other people, such as a therapist. In contrast, the broader *superordinate context* includes more distal events as well as enduring characteristics of the person and (social) environment. This may include personality traits, beliefs, abilities, psychological resources and problems, life circumstances, social relationships, and cultural factors. In psychedelic therapy, the broader treatment framework—including preparatory and integration sessions, pre- and postdosing interventions, the therapeutic relationship, and the ongoing psychotherapy process—constitutes a key component of the superordinate context. According to the contextual–experiential model (Figure 1), the role of any psychotherapeutic framework in psychedelic therapy is to purposefully shape these contextual conditions. Regardless of the specific therapeutic approach, these conditions should (a) increase the likelihood of acute, psychotherapeutically valuable experiences and (b) promote their successful integration into sustained and beneficial psychological change.

Belief Relaxation and Modification: Psychedelics Facilitate Psychological Change by Destabilizing Fixed Patterns of Mental Activity

According to contemporary theories of psychedelic effects, the pronounced context sensitivity characteristic of psychedelic states results from a temporary relaxation of otherwise relatively fixed cognitive patterns reflecting implicit assumptions about the world and self (Carhart-Harris & Friston, 2019; Carhart-Harris et al., 2023). However, rather than exclusively relaxing prior beliefs, psychedelics are perhaps better understood to alter beliefs through dynamic interactions involving both relaxation and reinforcement, influenced by contextual factors, dosage, and individual predispositions (Safron et al., 2025). From a psychotherapeutic standpoint, belief relaxation and modification hold significant relevance, as maladaptive implicit beliefs and negative expectancies are fundamental to psychological problems (Rief et al., 2015).

In dynamical systems theory, maladaptive cognitive and behavioral patterns can be conceptualized as entrenched *attractors*—overly reinforced states that the system habitually adopts (Carhart-Harris et al., 2023; Hipólito et al., 2023). The attractor concept applies to psychopathological states that characterize mental disorders (e.g., rumination in depression or drug craving in addiction) as well as underlying traits that constitute vulnerability factors for mental illness (e.g., excessive avoidance of emotions).[1] When mental

[1] From the perspective of Grawe's consistency theory (discussed below), psychopathological features of mental disorders constitute *disorder attractors*, whereas personality traits posing vulnerability factors are understood as *motivational attractors* (also referred to as *motivational schemas*).

activity is conceived as movement between states on a dynamical landscape (see Figure 2), these pathologically stable patterns correspond to steep basins of attraction that are not easily "escaped" from. Psychedelics are proposed to act by transiently flattening the curvature of the dynamical landscape, thereby relaxing these attractor states and potentially facilitating the formation or strengthening of more adaptive (or maladaptive) attractors (Carhart-Harris & Friston, 2019; Hipólito et al., 2023; Safron et al., 2025). This relaxation of attractor basins facilitates greater freedom of mental movement and increased responsiveness to internal and external perturbations, underpinning the heightened context sensitivity characteristic of psychedelic states.

Figure 3 illustrates two distinguishable types of longer term psychological changes that psychedelic-induced belief relaxation and modification may facilitate. First, freer movement within an attractor basin may gradually broaden it, potentially leading to bifurcations—that is, splitting the attractor into separate basins. Psychologically, this may correspond to incremental transitions from maladaptive cognitive and behavioral patterns toward healthier alternatives, reflecting gradual shifts and differentiation in underlying beliefs. Second, psychedelics may enable escape from entrenched attractors and the formation of entirely new attractor states. This process may correspond to sudden insights, novel perspectives, and profound belief revisions commonly reported during psychedelic experiences (McGovern et al., 2024).

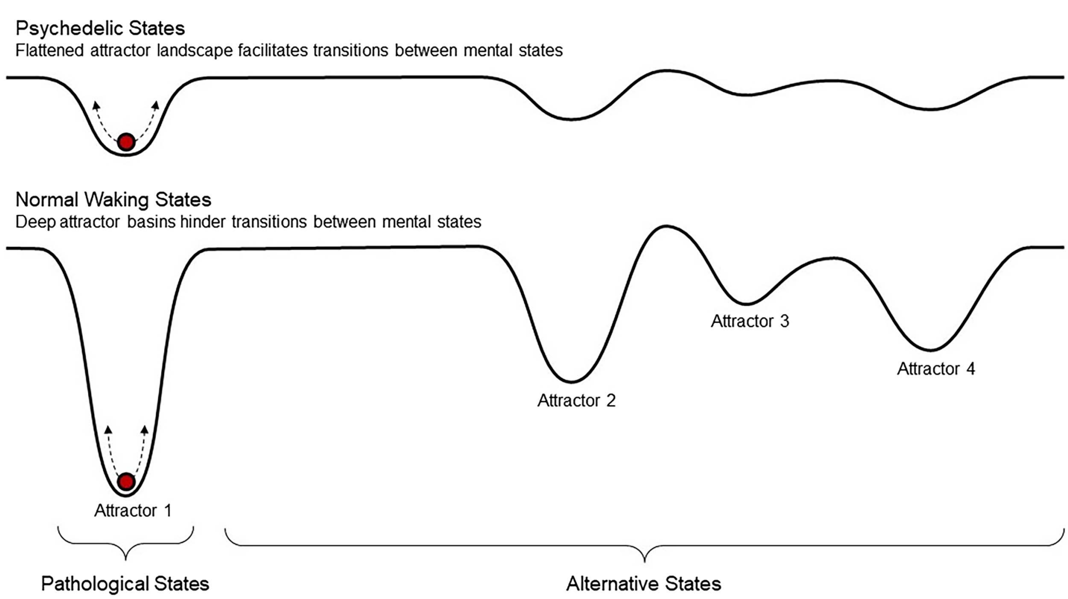
In summary, the concept of psychedelic-induced belief relaxation and modification seems to provide a plausible model of how these substances may facilitate different types of longer term psychological change. However, without further specifications, this model appears too abstract to inspire practice-oriented research questions or inform the clinical practice of psychedelic therapy. What exactly are the "pathological attractors" whose temporary destabilization is assumed to promote therapeutic change? What psychological processes are required for such change to unfold, and how can therapists influence these processes in a targeted manner? To be of practical use, a psychotherapeutic understanding of psychedelic therapy must provide concrete answers to such questions. It therefore seems necessary to integrate the belief relaxation model within a compatible psychological theory of psychotherapy.

Consistency Theory

Consistency theory (Grawe, 2004) was developed by the influential psychotherapy researcher Klaus Grawe (1943–2005) as a comprehensive transtheoretical model of human mental functioning, disorder, and psychotherapeutic change. Anchored in dynamical systems theory and offering concrete answers to questions left open by the belief relaxation model, consistency theory provides a well-suited bridge between psychedelic and psychotherapy research. A schematic illustration of central constructs is provided in Figure 4. As the overarching systemic principle of mental functioning, but also central to the emergence and maintenance of mental disorders, consistency theory assumes the organism's tendency to strive for

Figure 2

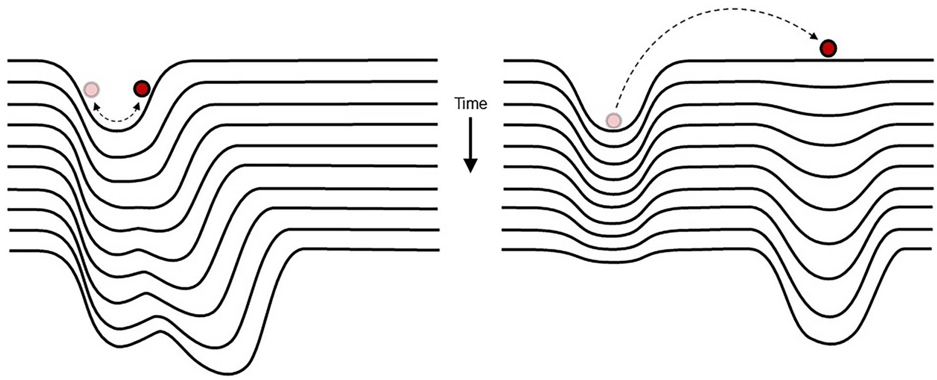
Psychedelic-Induced Belief Relaxation From a Dynamical Systems Perspective (Carhart-Harris et al., 2023; Hipólito et al., 2023)



Note. The system's state (represented by the horizontal position of the red ball) gravitates toward certain fixed points (attractors). Psychedelics are assumed to temporarily flatten the attractor landscape, facilitating movement within and between attractor basins, and thereby increasing the system's susceptibility to perturbations from internal or external context factors. Pathological mental states that have become entrenched are conceptualized as steep attractor basins that may be more easily escaped from in psychedelic states (under favorable contextual conditions). See the online article for the color version of this figure.

Figure 3

Two Broadly Distinguishable Types of Psychological Change That May Be Facilitated by Flattened Attractor Landscapes in the Sense of Psychedelic-Induced Belief Relaxation



Note. Facilitated movement along the slopes within an attractor basin leads to its gradual broadening and differentiation (left panel). Facilitated escape from one attractor basin leads to the formation of another basin (right panel). The two scenarios correspond to the consistency-theoretical concepts (introduced below) of schema assimilation and accommodation, respectively. See the online article for the color version of this figure.

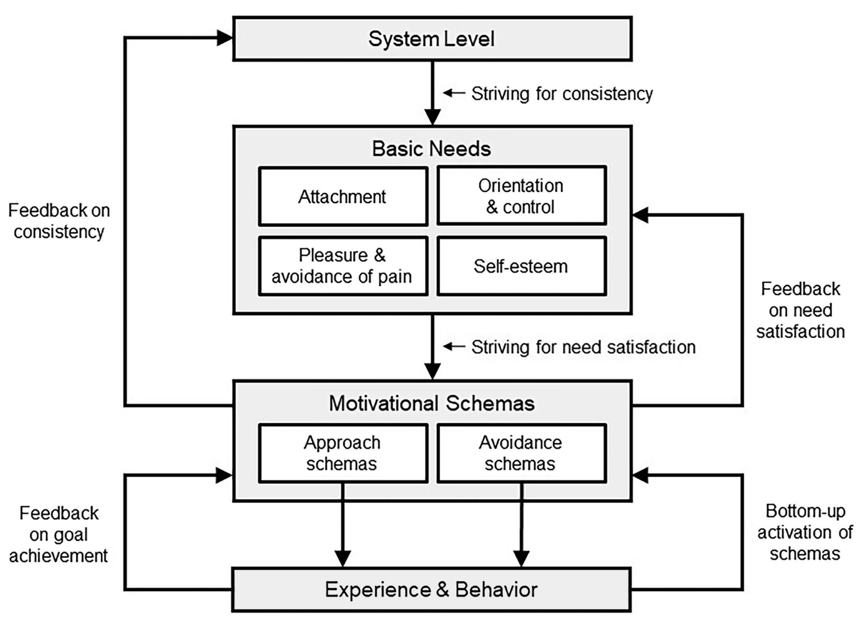
consistency between the multitude of simultaneous (and sometimes conflicting) intrapsychic processes that take place at any given moment.

Basic Needs and Motivational Schemas

An assumed primary purpose of this *consistency principle* is to reduce intrapsychic conflicts (inconsistencies) and thus ensure harmonious cofunctioning among the mental processes that serve the individual's *basic psychological needs*, that is, essential needs that have evolved phylogenetically and are shared by all humans. Consistency theory formulates four basic needs (Epstein, 1990; Grawe, 2004): (1) a need for *orientation and control*, that is, the need to understand, predict, and influence events in one's (social) environment; (2) a need for *attachment*, that is, the need to maintain deep emotional relationships with significant others; (3) a need for *pleasure and avoidance of pain*, that is, the need to seek pleasant states and avoid unpleasant ones; and (4) a need for self-esteem, that

Figure 4

Basic Concepts and Assumptions of Consistency Theory (Grawe, 2004)



Note. From a dynamical systems perspective (see Figure 2), motivational schemas are also referred to as motivational attractors.

is, the need to see oneself as "good" or "okay" and to receive corresponding social feedback. To serve these needs, each individual develops a variety of idiosyncratic *motivational schemas*. This refers to relatively stable cognitive-emotional–behavioral patterns whose purpose is to either satisfy (*approach schemas*) or protect a basic need (*avoidance schemas*). Figure 5 provides a simplified overview of the psychological components of exemplary approach and avoidance schemas. Grawe developed his concept of motivational schemas by integrating schema theory (Bartlett, 1932; Piaget, 1977) with neurobiological perspectives, dynamical systems theory, and his motivation-based psychotherapeutic approach. Accordingly, motivational schemas are conceived as manifestations of the past activations of more or less ingrained neural activation patterns. Whenever a motivational schema is activated, characteristic tendencies of perception, emotion, thought, and behavior emerge. Schema activation is also thought to be necessary for schemas to change: Schema *assimilation* occurs whenever an experience is integrated into an existing schema, which thereby becomes more ingrained and more differentiated. Schema *accommodation* occurs when a new experience cannot be assimilated by existing schemas, thus giving rise to new activation patterns that can, with repetition, form a new schema (Piaget, 1977). Adopting the same concept from dynamical systems theory as current models of psychedelic drug effects, consistency theory refers to motivational schemas also as *motivational attractors* (Thelen & Smith, 1998). Accordingly, assimilation and accommodation correspond to the two types of change in dynamical attractor landscapes that are illustrated in Figure 3.

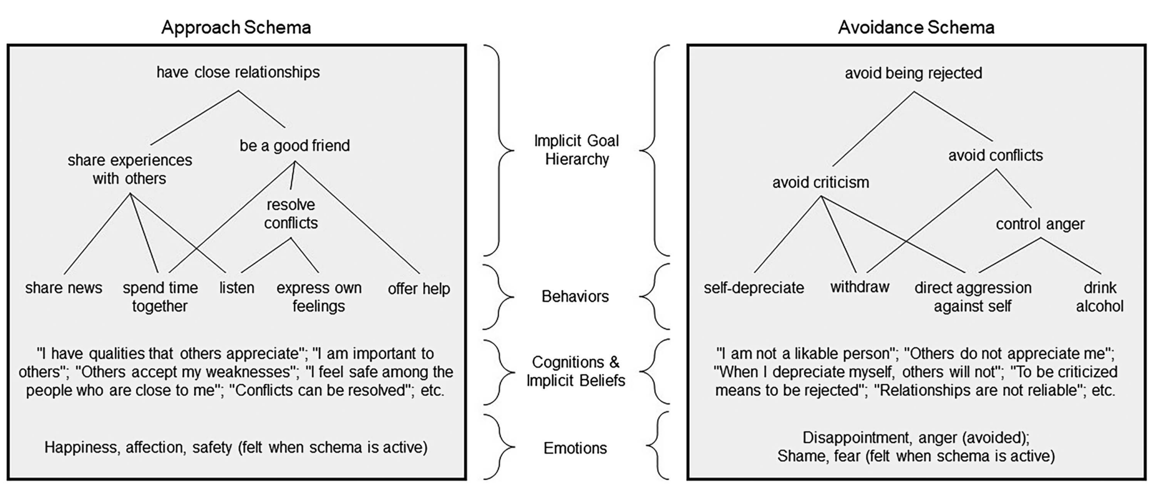
The usefulness of the schema concept in understanding mental functioning, dysfunction, and psychological change is evident in its adoption by various cognitive-behavioral (Beck, 1979; Goldfried & Robins, 1983; Safran & Segal, 1996; Young, 1990), humanistic (Greenberg et al., 1993; Sachse, 2004), and psychodynamic models of psychotherapy (L. Horowitz, 1994; M. J. Horowitz, 1988; Wachtel, 1980). An important difference between Grawe's model and other schema-theoretical conceptualizations is the assumed central role of motives. As exemplified in Figure 5, each schema is built around the pursuit of hierarchically organized approach or avoidance motives (goals) that the individual has developed to satisfy or protect a basic psychological need. This motivational component is thought to direct the development of the schema based on perceptions of need satisfaction and (threat of) need violation as the individual interacts with the social environment. As a motivational schema develops, the goal hierarchy assimilates new possibilities of goal attainment and thereby becomes increasingly differentiated. The psychological structures and patterns that are selected in ontogenetic development are the ones that effectively establish congruence between approach/avoidance goals and perceptions of goal attainment. Developmental conditions encountered in early life are considered critical in this regard: Favorable conditions that provide ample opportunities for need-satisfying experiences are assumed to promote well-differentiated approach schemas. Conversely, unfavorable conditions where basic needs are often threatened or violated give rise to a dominance of avoidance schemas that often persists throughout the individual's lifespan (Grawe, 2004).

Mental Disorders and Psychotherapeutic Change

Excessive avoidance motivation is evident when avoidance schemas are so deeply ingrained and highly differentiated that they dominate mental activity across broad areas of the individual's life. Two related forms of psychological inconsistency that characterize mental functioning in such individuals are excessive *discordance* and *incongruence*. Inhibited by discordant (conflicting) avoidance schemas, underdeveloped approach schemas are rarely activated and thus remain weakly ingrained and undifferentiated. The example in Figure 5 points to approach/avoidance schema conflicts that may arise in social situations concerning the need for attachment. Of note, discordant schemas may also pertain to different

Figure 5

Psychological Components of Exemplary Approach and Avoidance Schemas (in This Case Serving the Basic Psychological Need for Attachment)



Note. Schema components not listed here for the sake of conciseness include the developmental conditions of schemas, schema-activating situations, situational appraisals, perceptions, memories, etc. (see Grawe, 2004).

psychological needs (e.g., approach schemas serving attachment needs may conflict with avoidance schemas protecting control needs). The second, related source of inconsistency is *incongruence* between motives and ongoing perceptions of goal attainment. When underdeveloped approach schemas are regularly inhibited by dominant avoidance schemas, the inherent approach goals remain constantly unfulfilled. The resulting chronic dissatisfaction of basic psychological needs is seen as a breeding ground for mental disorders, which are thought to emerge as a consequence of maladaptive attempts at reducing inconsistency. Adopting again the dynamical systems perspective, consistency theory assumes that *disorder attractors* emerge as autonomous determinants of mental functioning, based on sustained inconsistency and an interplay between genetic, individual, social, and cultural dispositions. Disorder attractors represent self-reinforcing maladaptive patterns of experience and behavior that are highly specific to the given type of disorder (e.g., social withdrawal, negative thoughts, and rumination in depression; fear and avoidance behavior in anxiety disorders; and drug craving in substance use disorders). It is assumed that disorder attractors, once established, become functionally independent from motivational attractors and constitute additional sources of inconsistency.

According to consistency theory, effective psychotherapy works by increasing consistency in mental functioning, thereby improving the satisfaction of patients' basic psychological needs. Since disorder attractors are considered autonomous determinants of mental functioning that, once established, contribute independently to psychological inconsistency, it is assumed that disorder-specific interventions aimed at destabilizing these attractors (e.g., fear exposure in panic disorder) are often required. However, for sustained success, therapy must in many cases also address the motivational sources of inconsistency that originally led to the emergence of the disorder. This usually requires a resolution of schema conflicts by strengthening underdeveloped approach schemas and weakening the respective discordant avoidance schemas. In summary, disorder attractors and maladaptive motivational attractors represent the primary targets of effective psychotherapy.[2] They plausibly correspond to the "pathological attractors" (Carhart-Harris & Friston, 2019; Hipólito et al., 2023) whose destabilization has been proposed as a mechanism of psychedelic therapy (see Figure 2).

Grawe's General Change Mechanisms

Having outlined a more differentiated idea of *what* is changed by effective psychotherapy (and, by implication, effective psychedelic therapy), it seems appropriate to elaborate on *how* such change is achieved. In this regard, Grawe proposed five general change mechanisms (GCMs) that characterize effective psychotherapy: (a) resource activation, (b) therapeutic relationship, (c) problem actuation, (d) clarification, and (e) mastery. These GCMs were identified based on a comprehensive review of hundreds of controlled therapy studies and naturalistic observations of the process–outcome relationship in psychotherapy (Grawe, 1997, 2004; Grawe et al., 1994). In current psychotherapy research, Grawe's GCMs and derived constructs are assessed using postsession questionnaires with corresponding versions for patient and therapist perspectives (Flückiger, Regli, et al., 2010; Mander et al., 2013), structured observer ratings of videotaped therapy sessions (Boyle et al., 2020; Flückiger et al., 2014; Gassmann & Grawe, 2006), and qualitative content analysis of patient interviews (e.g., Wucherpfennig et al., 2020). GCM measures have been used for various purposes, such as studying process–outcome associations (e.g., Brockmeyer et al., 2023; Flückiger et al., 2014; Grosse Holtforth & Flückiger, 2012; Mander et al., 2013, 2014; Stangier et al., 2010; Wrede et al., 2023b; Wucherpfennig et al., 2020; Znoj et al., 2010), predicting psychotherapy dropout (Kegel & Flückiger, 2015), comparing therapeutic modalities (e.g., Stangier et al., 2010), evaluating associations between therapy process and observed context factors (e.g., Edelbluth et al., 2024; Prinz et al., 2021), testing effects of experimental interventions on the therapy process (e.g., Flückiger & Grosse Holtforth, 2008; Flückiger et al., 2009; Gassmann & Grawe, 2006), and exploring session-by-session dynamics (e.g., Moggia et al., 2023; Rubel et al., 2017; Schilling et al., 2021; Smith & Grawe, 2005; Tschacher et al., 2000; Wrede et al., 2023a).

Grawe's GCM model is consistent with the view that much of the effectiveness of various psychotherapeutic practices is attributable to "common factors," that is, shared underlying principles of change that can be harnessed by superficially distinct therapeutic methods, irrespective of theoretical orientation (Cuijpers et al., 2019). Other conceptualizations of common factors that have been discussed in relation to psychedelic therapy are Jerome Frank's "effective features" (Frank & Frank, 1993) and Bruce Wampold's "contextual model" of psychotherapy (Wampold, 2015). As recently noted (Gukasyan & Nayak, 2022; Nayak & Johnson, 2021), the four effective features of psychotherapy postulated by Frank—(a) a therapeutic relationship, (b) a healing setting, (c) a rationale, conceptual scheme, or myth, and (d) a ritual—are clearly present in psychedelic therapy and likely contribute to its effects. Similarly, Greenway et al. (2020) have argued that the three common factors at the center of Wampold's model—(a) the "real relationship," (b) the creation of expectations through explanation of disorders and treatments, and (c) the enactment of health-promoting actions—correspond to long-standing approaches to psychedelic therapy. Compared to these compatible common factor models, Grawe's GCMs offer a more differentiated account of the psychological mechanisms and associated therapeutic experiences that characterize effective psychotherapy processes. This experiential focus renders the GCM model well applicable to the centerpiece of psychedelic therapy, that is, the psychedelic experience. Hence, together with the overarching consistency theory, the GCM model offers a transtheoretical psychotherapeutic understanding of distinct therapeutic qualities of psychedelic experiences. In accordance with this, the contextual–experiential model (Figure 1) assumes that GCM-related experiences are important mediators of psychedelic-occasioned psychological change.

Another advantage of the GCM model is that psychotherapeutic interventions can be mapped to the GCMs that they target (Tschacher et al., 2014; Wucherpfennig et al., 2024). According to the contextual–experiential model (Figure 1), it should be assumed

[2] Besides motivational and disorder attractors, consistency theory assumes two further semi-autonomous determinants of mental functioning that are relevant to psychotherapy but are not considered here for the sake of brevity: *emotional attractors* (once triggered, strong emotions can transiently take over control from the motivational attractors that usually determine experience and behavior) and *interpersonal attractors* (interpersonal relationships cannot be unilaterally influenced at will and are therefore considered partially independent determinants).

that therapeutic qualities of patients' psychedelic experiences correspond with therapeutic interventions that are implemented in the superordinate context. For example, it can be hypothesized that implementing clarification-oriented interventions in preparatory sessions can increase the occurrence of clarification experiences in dosing sessions, and that successful integration of these experiences is, in turn, facilitated by again implementing clarification-oriented interventions in integration sessions. The GCM model thus suggests testable links between therapeutic interventions, experiences, and outcomes that are of direct relevance to the clinical practice of psychedelic therapy.

GCMs in Psychotherapy and Psychedelic-Occasioned Psychological Change

In the following sections, each of the five GCMs is briefly defined and then examined in relation to psychedelic experiences and psychedelic therapy. Building on the contextual–experiential model (Figure 1) and examining extensive evidence from clinical and nonclinical psychedelic research, we aim to explain the role of Grawe's five GCMs in psychedelic therapy as follows:

- GCM-related psychedelic experiences occur frequently in psychedelic therapy and other contexts.
- GCM-related psychedelic experiences are important mediators of psychedelic-occasioned psychological change and contribute substantially to the efficacy of psychedelic therapy.
- The occurrence of GCM-related psychedelic experiences is context-dependent. Preparatory and dosing sessions in psychedelic therapy can be understood as the systematic effort to establish contextual conditions that are conducive to the occurrence of GCM-related psychedelic experiences. Common interventions in preparatory and dosing sessions can be mapped to GCMs.
- Longer term effects of GCM-related psychedelic experiences are also context-dependent. Integration sessions in psychedelic therapy can be understood as the systematic effort to establish contextual conditions that are conducive to the successful integration of GCM-related experiences. Common interventions in integration sessions can also be mapped to GCMs.

To clarify these points, we draw links between GCMs and acute psychedelic drug effects, current concepts in psychedelic process–outcome research, and common practices in psychedelic therapy. We consider quantitative empirical evidence from psychedelic therapy trials, experimental studies in healthy volunteers, and observational studies of naturalistic psychedelic use. To illustrate conceptual considerations, we quote qualitative research with patients and other psychedelic users. For conciseness, we limit our discussion to classical psychedelics such as psilocybin, LSD, and the dimethyltryptamine-containing herbal preparation *ayahuasca*. However, many of our considerations may also apply to "atypical psychedelics" (see Mitchell & Anderson, 2024) such as the dissociative ketamine or the entactogen 3,4-methylenedioxymethamphetamine (MDMA).

Resource Activation

The term *resources* encompasses all available means a person might use to pursue approach motives and satisfy his or her *basic psychological needs* (i.e., orientation and control, attachment, pleasure, and self-esteem; Epstein, 1990; Grawe, 2004). Resources thus include a person's positive goals, related positive beliefs about the world and self, personal abilities, and other positive attributes such as possessions or functioning relationships. This very broad definition of resources is contrasted with a narrower definition of the GCM resource activation, which refers to the identification, mobilization, and utilization of resources. Therapeutic interventions targeting this GCM thus focus on the patient's existing strengths and healthy qualities. This is thought to establish positive feedback circuits that reinforce positive expectations, reduce demoralization, foster the therapeutic relationship, and increase openness toward therapeutic interventions (Flückiger, Wüsten, et al., 2010). Effective resource activation thereby mobilizes the patient's approach-motivational system as a driving force for therapeutic change. Through repeated need-satisfying experiences, driven by self-reinforcing processes of reward learning and self-efficacy, hitherto underdeveloped approach tendencies can develop into fully grown approach schemas, and existing approach schemas can become strengthened and more differentiated. From a dynamical systems perspective (Figure 2), resource activation describes a process by which attractors corresponding to approach schemas are deepened and broadened, making them more stable and likely to "capture" the individual's psychological state. The ensuing relative strengthening of approach versus avoidance motivation is assumed to reframe negative emotions associated with the closely intertwined GCM problem actuation (discussed below): They are transformed from unwelcome experiences to something that must be actively confronted to achieve positive goals (Grawe, 1997, 2004). It is therefore considered important that effective resource activation be already implemented early in the therapy process.

Resource Activation in Psychedelic Therapy and Psychedelic States

Resource-Activating Interventions in Preparatory Sessions. Consistent with this view, preparatory sessions in psychedelic therapy commonly involve resource-activating interventions such as fostering realistically positive treatment expectancy (Nayak & Johnson, 2021; Thal, Engel, & Bright, 2022). A related standard intervention is the discussion of intentions for an upcoming psychedelic experience during preparatory sessions (Thal, Wieberneit, et al., 2022). By establishing positive expectancies related to personally meaningful approach goals, intention-setting can be assumed to increase patients' readiness to engage openly with the upcoming experience, including potentially challenging aspects. According to our contextual–experiential model (Figure 1), successful resource activation in preparatory sessions can be assumed to promote the occurrence of resource-activating psychedelic experiences in dosing sessions. However, whereas active shaping of approach goals is among the most studied resource-activating interventions in psychotherapy research (DeFife & Hilsenroth, 2011; Wollburg & Braukhaus, 2010), the effects of fostering positive expectancy and setting intentions on patients' psychedelic experiences remain to be systematically investigated.

Resource-Activating Psychedelic Experiences.

Mystical Experiences. That the acute psychedelic experience mediates positive longer term outcomes (see Figure 1) is relatively well established with regard to resource-activating experiences. Many patients treated with psilocybin and healthy research volunteers rate their psychedelic experiences as among the most personally meaningful experiences of their entire lives (Johnson et al., 2019). This phenomenon is linked to so-called *mystical experiences*, that is, experiences described as ineffable and characterized by positive emotionality (joy, awe, amazement, peace, etc.), a sense of unity or connectedness, and transcendence of time and space. Clinical studies testing psychedelic therapy for various mental disorders have found that these experiences, measured using the Mystical Experience Questionnaire (MEQ; Barrett et al., 2015) and the strongly correlated Oceanic Boundlessness Scale of the Altered States of Consciousness Rating Scale (r = .93; Liechti et al., 2017), positively predict treatment outcomes (Ko et al., 2022; Romeo et al., 2025). In healthy volunteers, mystical experiences likewise predict positive longer term outcomes such as increased well-being, life satisfaction (McCulloch et al., 2022; Schmid & Liechti, 2018), and trait mindfulness (Søndergaard et al., 2022). The following report of a "complete mystical experience" (i.e., a score of >60% on all MEQ subscales; Barrett et al., 2015) illustrates the resource-activating quality of such experiences:

> The feeling of joy and love was the energy in the universe, completely intense and multiplied by 100. It was like taking those two emotions and concentrating them, to have them in their purest form without any worries or other troubles that can come with everyday reality. Everything else ceased to matter while those two emotions were so pure—they made everything incredibly beautiful. When I was soaring through the soundwaves with this energy, I could see all the people close to me in my life appear. My partner, my sister and her boyfriend and their new-born son, my mother, my amazing friends. I felt immensely privileged to be part of this universe/community, to be able to feel those feelings in such a pure form, to have such deep emotions in my body—and I was overwhelmed with gratitude for this world. That everything simply is. And with that I was overtaken by a desire to protect it all, to show the world how beautiful it is and to take care of it. (McCulloch et al., 2022) [healthy volunteer; psilocybin]

Several features of resource-activating experiences are evident in this report: First, intense positive emotions and the absence of avoidance-motivated mental activity ("without any worries or other troubles that come with everyday reality") indicate a strongly approach-motivated mode of mental functioning. This is associated with satisfaction of all four basic psychological needs: pleasure ("those two emotions were so pure—they made everything incredibly beautiful"), orientation ("everything else ceased to matter"), attachment ("I could see all the people close to me in my life appear"), and self-esteem ("I felt immensely privileged"). Not least, resource activation is evident in emergent positive goals ("a desire to protect it all, to show the world how beautiful it is and to take care of it"), illustrating that mystical experiences can provide a developmental impetus for the formation, strengthening, and differentiation of approach schemas.

Resource Activation Beyond the "Mystical Experience" Construct. Although the therapeutic quality of mystical experiences is best understood in terms of the GCM resource activation, it is important to note that not all resource-activating psychedelic experiences are necessarily mystical-type experiences. The defining features of resource activation (approach-motivated mental activity, positive emotionality, satisfaction of basic needs, and emergence of approach goals) are conceptually and empirically dissociable from the mystical experience construct. In a sample of naturalistic psychedelic users, the Goals and Adaptive Patterns Insights subscale of the Psychological Insight Questionnaire (PIQ; Davis et al., 2021), which measures resource-related insights (e.g., "Experienced validation of my life, character, values, or beliefs"; "Awareness of beneficial patterns in my actions, thoughts, and/or feelings"; "Experienced validation of my life, character, values, or beliefs"), was only moderately correlated with MEQ scores (r = .50). Less narrowly focused on psychological insight, the Resource Activation scale of the recently developed General Change Mechanisms Questionnaire (GCMQ; Wolff et al., 2024) is designed to capture a broader range of experiences characterized by positive emotionality, positive appraisals of the world and self, and satisfaction of psychological needs (e.g., "I could see myself in a positive light"; "I felt connected to things that give my life meaning"; "I felt in contact with my strengths"). Although this more comprehensive operationalization of resource activation shows a somewhat stronger correlation with Oceanic Boundlessness (r = .67), these findings underscore the need to look beyond the mystical experience—a construct rooted in the psychology of religion—to fully appreciate the diversity of resource-activating psychedelic experiences. The only partial overlap with resource activation may help explain why correlations between MEQ scores or related measures and mental health outcomes do not always replicate (e.g., Calder et al., 2024; Sloshower et al., 2023). Further explanations include the view that resource activation must be combined with problem actuation (see below) to achieve corrective experiences and exert sustained therapeutic effects, and that longer term effects of psychedelic experiences may be substantially dependent on superordinate context factors, as we discuss in the following.

Integration of Resource-Activating Psychedelic Experiences.

Increased Approach Motivation. Following a resource-activating psychedelic experience, further strengthening and differentiation of emergent approach motives should be assumed to require repeated reactivation in the context of need-satisfying interactions with the environment. In the process of integrating a resource-activating psychedelic experience, all of the patient's existing resources—positive motives, abilities, and also the environment's need-satisfying potentials—can be regarded as favorable superordinate context factors (see Figure 1). An important purpose of integration sessions is to utilize these resources to promote approach-motivated behaviors so that as many new need-satisfying experiences as possible can occur and more stable approach schemas can form. In line with this, standard protocols for integration sessions aim at fostering long-term habits and activity patterns congruent with the therapeutic intent of the treatment (Garcia-Romeu & Richards, 2018).

Various forms of psychedelic-occasioned psychological change observed in clinical settings and beyond can be interpreted as signs of strengthened approach motivation, including increased altruism (Aday et al., 2020), spirituality (Griffiths et al., 2006, 2008, 2018), nature-relatedness (Kettner et al., 2019), and social connectedness (Kettner et al., 2021). The following patient report, describing positive shifts in a close relationship, illustrates how such increases in approach-motivated mental functioning may, under favorable circumstances, initiate a self-reinforcing process of resource activation.

> I was appreciating the pureness of her, if you will, her being, in a way. In a way, I had not allowed myself, or not realized, or whatever the case was. And so that was pretty much immediate, but I felt like that had a lasting effect. I feel that kind of changed my perception of our relationship over the ensuing weeks in a way. It allowed me to be more open with her, express my appreciation a lot for her in way that I had not. I think it had a positive effect—kind of a bit of a snowball effect. (Belser et al., 2017) [depression/anxiety associated with a cancer diagnosis; treated with psilocybin]

Such processes may be facilitated by strengthened reward learning under acute effects of psychedelics, which may lead to subsequent perseveration on reversal learning (Kanen et al., 2023) and set-shifting tasks (Pokorny et al., 2020). Following a psychedelic experience, increased shielding of emergent approach goals may often pose a therapeutic advantage, protecting continued resource activation against demotivating influences of failures, setbacks, or conflicting avoidance schemas. On the other hand, it might also facilitate the persistent pursuit of maladaptive approach goals despite negative consequences. This risk should be taken into account, especially when emergent approach motives concord with dominant avoidance schemas, which are then reinforced rather than challenged. While likely idiosyncratic, such maladaptive forms of postpsychedelic perseverance may be reflected in circumscribed patterns of unfavorable integration like *spiritual bypassing* (the use of spiritual beliefs or practices to avoid unresolved psychological issues; Cashwell et al., 2007) and *spiritual superiority* (the belief that one's spiritual insights confer inherent superiority over others; Vonk & Visser, 2021). More generally, these concerns underscore the need to consider not only the therapeutic potential but also possible adverse consequences of positive (e.g., mystical-type) psychedelic experiences and to examine the contextual conditions that may influence these outcomes.

Increased Motivation to Confront Problems. Ideally, emergent approach goals and the broader resources that surround them are discordant with existing psychological problems. When newly salient motives signal the opportunity for better need satisfaction, this may entail a willingness to engage with emotional problems that would otherwise be avoided (see our discussion of the GCM problem actuation below). Resource-activating psychedelic experiences may support this process through the vivid activation of latent values or aspirations, thereby reframing the confrontation of psychological difficulties as meaningful and worthwhile. The following report of an ayahuasca experience by a woman with anorexia nervosa illustrates such a shift.

> I saw myself as a rotting, decaying skeleton and then I saw myself as this beautiful full-bodied, just beautiful woman with this long hair, and I, like, I wanted to be that woman. I wanted to be that full, loving woman that has so much to offer my family and world. It was, and then I felt my ribs and I could feel them, they were so hollow and I was just, I was like, I can't wait to get back and just start gaining some weight. (Lafrance et al., 2017) [eating disorder; treated with ayahuasca]

Recognizing the motivational potential of resource-activating psychedelic experiences, several experimental therapy protocols have been developed that aim to support and channel such shifts, including motivational enhancement therapy for alcohol use disorder (Bogenschutz et al., 2015, 2022) and acceptance and commitment therapy for depression (Sloshower et al., 2020; Watts & Luoma, 2020).

Therapeutic Relationship

The therapeutic relationship is an important predictor of psychotherapy outcomes (Baier et al., 2020). A central aspect for which especially strong evidence exists is the *therapeutic alliance*, including a positive emotional bond between patient and therapist as well as mutual agreement on goals and tasks (Flückiger et al., 2018). Like all GCMs, the therapeutic relationship is understood as a dynamic process that depends on other GCMs and feeds back into them. A particularly close mutual dependence is assumed between the GCMs therapeutic relationship and resource activation (Grawe, 1997): On condition that the therapist manages to effectively activate the patient's resources, the relationship itself can become a powerful new resource. According to the principles of shaping a *complementary relationship* (Caspar et al., 2005), an optimal alliance is achieved when the therapist provides the patient with ample opportunities for need-satisfying experiences within the patient–therapist interaction. To direct this process in an individually tailored fashion, the therapist must understand what motivational schemas the patient has developed to satisfy and protect his or her basic psychological needs in relationships and, even when avoidance motives dominate, infer and attend to the patient's most important positive relationship goals. By understanding these as resources and continuously adjusting his or her behavior in a complementary fashion, the therapist lays the necessary groundwork for favorable *self-relatedness* (Orlinsky & Howard, 1987), that is, "active collaboration and the willingness and ability to open up, to let in that which is new, and to experiment" (Grawe, 1997).

The Therapeutic Relationship in Psychedelic Therapy and Psychedelic States

It is presumably no coincidence that Grawe's circumscription of favorable self-relatedness resonates with the much-referenced assertion that the ability to "trust, to let go, and to be open to whatever comes" (Pahnke, 1969) is crucial when undergoing a psychedelic experience. While the therapeutic relationship has indeed always been considered a fundamental aspect of psychedelic therapy (Abramson, 1967; Nayak & Johnson, 2021), the extent of its role has also been a topic of debate. In line with the psychoanalytic tradition that prevailed in the mid-20th century, many scholars saw the primary therapeutic function of psychedelic drugs in their role as catalysts of patient–therapist relationship dynamics. This view was not universally held, however.

> Is LSD a therapy in itself, with the therapeutic relationship functioning merely for support? I believe this is so. Or is the essential therapeutic process the sort of thing Dr. Abramson described, in which the therapeutic relationship is the essence of the therapy, and LSD simply facilitates or catalyzes it? These questions underlie a good deal of the discussion here, and are perhaps of more importance than we have recognized. (Lennard & Hewitt, 1960, quoting a statement by conference panelist Robert Murphy)

Shaping the Therapeutic Relationship Is a Primary Focus of Preparatory Sessions in Psychedelic Therapy. Although transference phenomena are still considered an important aspect of psychedelic therapy (Koslowski et al., 2021), contemporary models place greater emphasis on the therapist's role in establishing trust and rapport prior to dosing sessions (Garcia-Romeu & Richards, 2018). Presence, trust, and empathy are considered key qualities of psychedelic therapists (Thal, Engel, & Bright, 2022). Standard preparatory interventions such as intention-setting and psychoeducation serve the alliance by establishing agreement on goals and tasks as well as confidence in therapists' trustworthiness and competence. Another routine practice of central importance to the therapeutic relationship is that therapists take time to inquire about the patient's biography and current life situation (Garcia-Romeu & Richards, 2018).

Therapeutic Relationship Affects the Quality of Patients' Acute Psychedelic Experiences, and Vice Versa. Clinical standard doses of psychedelics often evoke a profound sense of defenselessness, disorientation, and loss of control (Johnson et al., 2008). Therefore, both the anticipation of such an experience and the experience itself often strongly activate the need for orientation/control as well as the need for attachment.

> It really hit me very strong. And um, it was terrifying. … I was completely disoriented. … I was really, maybe, in the hold of a ship at sea. Rocking. Absolutely nothing, nothing to anchor myself to, nothing, no point of reference, nothing, just lost in space, just crazy, and I was so scared. And then I remembered that Tony and Michelle were right there and suddenly realized why it was so important that I get to know them and they to get to know me. And reached out my hand and just said "I'm so scared." And I think it was Tony who took my hand … and said "It's all right. Just go with it. Go with it." And um, and I did. (Belser et al., 2017) [depression/anxiety associated with a cancer diagnosis; treated with psilocybin]

According to our contextual–experiential model (Figure 1) and the principle of complementary relationship design (Caspar et al., 2005; Grawe, 2004), we assume that such favorable, need-satisfying relational experiences are most likely to occur within a complementary therapeutic relationship that align with the individual's functional motives for orientation/control and attachment.

Direct evidence supporting the view that the previously established therapeutic relationship affects the therapeutic quality of patients' acute psychedelic experiences was presented by Murphy et al. (2022): In a clinical trial testing psilocybin-augmented psychotherapy for depression, the strength of therapeutic alliance measured 1 day before dosing sessions predicted reduced depression symptoms posttreatment. Consistent with the contextual–experiential model (Figure 1), the effect of therapeutic alliance (a superordinate context factor) on treatment outcome was sequentially mediated via presession rapport (a more immediate context factor) and psilocybin-occasioned mystical-type (i.e., resource-activating) experiences as well as *emotional breakthrough experiences*. The latter, which involve a broader synergistic action of GCMs (see our discussion under the Problem Actuation and Mastery sections), in turn positively predicted the therapeutic alliance weeks later. Levin et al. (2024) likewise found positive correlations between participant-rated therapeutic alliance, therapeutic experiences (mystical experience and psychological insight) in dosing sessions, and long-term reduction in depression symptoms following psilocybin-augmented psychotherapy. Beyond the domain of classical psychedelics, there is emerging evidence that the therapeutic alliance contributes to treatment outcomes in MDMA-assisted psychotherapy for posttraumatic stress disorder (Zeifman et al., 2024). A recent meta-analysis that pooled all published alliance–outcome associations in psychedelic therapy (six effect sizes across the three trials mentioned here; two psilocybin and one MDMA) found an overall correlation of $r = .41$, corresponding to an effect size of about 0.90—marginally larger than the drug versus placebo effect of psilocybin on anxiety and depression symptoms (Flückiger et al., 2025). Nevertheless, larger samples and more elaborate observational and experimental studies are needed to test the assumption that the therapeutic relationship interacts with psychedelic drug effects, rather than simply exerting additive influences on acute experiences and treatment response.

Therapeutic Relationship and Illusory Social Events in Psychedelic States. Following standard protocols for dosing sessions (where participants are encouraged to introspect rather than interact), direct verbal communication is usually limited during much of the acute drug effect. However, due to the context sensitivity that characterizes psychedelic states (Figure 1), nonverbal behaviors of therapists and their mere presence are nonetheless assumed to influence the quality of the psychedelic experience. Corresponding phenomena can be observed in *illusory social events*, that is, scenic experiences of social interaction that occur within introspection.

> My first experience today seemed to originate out of Brian covering me with the blanket. This gradually transformed into the vision and sense that Bill and Brian (the guides) and I were covered by a blanket-like net that linked us together emotionally and mentally. (Richards & Cosimano, 2013) [healthy volunteer; psilocybin]

While this example seems to directly represent the participant's relationship with the attending guides, less direct representations are also conceivable. An intriguing possibility is that representations of a previously established patient–therapist relationship may emerge in the form of illusory social events involving "psychedelic entities", that is, pseudohallucinatory beings that are perceived to possess personhood, agency, and intelligence (Lutkajtis, 2020). Descriptions of such entities often indicate qualities that would be considered favorable features of real psychotherapists, such as empathy, positive regard, trustworthiness, and competence.

> This teacher student relationship came about, so although I knew it was going on in my head, it was as if there was somebody there giving me prompts, think about this or that, it could weave its way and drop you straight into an image. (Watts et al., 2017) [depression; treated with psilocybin]

Potential therapeutic roles of such entity encounters have been considered (Lutkajtis, 2020). In this regard, it seems important that the perceived behavior of psychedelic entities can appear to be complementary to patients' interpersonal motives. In the following example, this may involve an implicit goal to acquire encouragement, appreciation, or affection by respected others, perhaps related to psychological needs for attachment and self-esteem.

> These voices were ancient, she was so eloquent. She kept calling me "my darling": "Life is to be lived my darling." She talked to me in such a loving way, all these things to say about my life. (Watts et al., 2017) [depression; treated with psilocybin]

Within a limited scope and time frame, illusory social events of this kind might fulfill similar therapeutic functions as a relationship with a complementarily behaving real therapist. While the therapeutic role of illusory social events remains to be studied systematically, a testable hypothesis based on our contextual–experiential model (Figure 1) is that complementary "behavior" of psychedelic entities reflects the actual behavior of the real therapists (or other individuals) who constitute the interpersonal context in which the experience takes place.

Problem Actuation

In effective psychotherapy, resource activation and the therapeutic relationship form the basis on which patients can be motivated to do the hard and often frightening work of changing problematic, entrenched patterns of experience and behavior. Almost all conceptualizations of psychotherapy share the view that such change requires what Grawe (1997) referred to as problem actuation: Patients must get into emotional contact with the problems that are to be changed. Different therapeutic traditions have developed a wide variety of strategies for problem actuation: exposure to feared stimuli in behavior therapy; experience-activating procedures such as imagination, chair work, or focusing in the humanistic-experiential therapies; transference work in psychodynamic therapies; inclusion of real interaction partners in couples, family, or group therapy; nonverbal approaches in music, arts and movement therapy, and so on. Problem actuation is thought to be necessary because maladaptive patterns of thought, emotion, and behavior can only be overridden when they are activated (Grawe, 2017). This principle aligns with research on extinction learning, which shows that learned fear responses must be reactivated in new contexts in order to be modified (Pittig et al., 2016). The related concept of *corrective experience* (i.e., new experiences that contradict the negative expectancies implicit in an actuated problem, resulting in relief, catharsis, or insight) is common across many traditions of psychotherapy and also extensively supported by empirical research (Goldfried, 2012). In this sense, any effective psychotherapy must arguably incorporate the core principles of exposure therapy: encouragement and support to confront what is otherwise avoided.

Problem Actuation in Psychedelic Therapy and Psychedelic States

Many historical and modern theoretical approaches to psychedelic therapy suggest that psychedelic states may provide uniquely conducive conditions for such exposure-like experiences (Roseman et al., 2019; Wolff et al., 2020). Conducting exposure therapy in a narrower sense during the acute psychedelic state (e.g., deliberate presentation of feared stimuli or guided engagement with traumatic memories) represents a promising direction for future research, particularly in light of emerging evidence that psychedelics may intensify initial fear responses, enhance extinction learning, and facilitate broader generalization of extinction effects (for discussion, see Doss, DeMarco, et al., 2024). Similarly oriented toward active problem actuation in dosing sessions, the historical model of "psycholytic therapy" combined talk therapy with lower dose psychedelics, aiming to evoke psychodynamic processes such as a softening of defenses, age regression, spontaneous childhood memories, and enhanced transference (Passie et al., 2022). In contrast to such more content-directive models, current standard protocols for psychedelic dosing sessions involve no direct problem-actuating interventions or active encouragement to engage with specific problem-related topics or stimuli. Nevertheless, intense problem-actuating psychedelic experiences often occur spontaneously as patients engage in introspection, likely arising from an interaction between context factors (discussed below) and specific drug effects (see Figure 1).

> I was crying a lot, crying with relief, and experiences, visions, memories were brought to me that I'd suppressed for a long time, and I was having to confront them. (Watts & Luoma, 2020) [depression; treated with psilocybin]

Various interrelated properties of acute psychedelic states appear conducive to spontaneous problem actuation, including increased semantic activation and associative processing (Family et al., 2016; Kraehenmann et al., 2015; Spitzer et al., 1996), emotional responsiveness (Preller et al., 2017; including sensitivity to the emotional quality of music, Barrett et al., 2018), and mental imagery (Kraehenmann et al., 2017; Stoliker, Preller, et al., 2025). Furthermore, as discussed below, psychedelics may under favorable conditions facilitate favorable forms of self-relatedness, including acceptance and self-compassion, thereby promoting patients' readiness to actively engage with psychological problems during (and after) dosing sessions (Wolff et al., 2020, 2022).

Problem Actuation and the "Challenging Experience" Construct. The construct of *challenging experience*, operationalized in the widely used Challenging Experience Questionnaire (Barrett et al., 2016), covers a broad range of distressful affective, cognitive, and physiological experiences that can occur during psychedelic states. The Challenging Experience Questionnaire correlates only moderately ($r = .50$) with the GCMQ Problem Actuation scale (Wolff et al., 2024; e.g., "I experienced feelings that I previously avoided"; "I was confronted with my problems"; "I could feel that my vulnerable spots were being touched upon"). Relatedly, the conceptual mapping between the challenging experience construct and psychotherapeutic processes is not straightforward. First, strong negative emotions (in psychedelic states and normal waking consciousness alike) do not always indicate problem actuation, which refers exclusively to the activation of psychological patterns that are actually problematic. Thus, challenging psychedelic experiences, especially under uncontrolled conditions, may often reflect healthy reaction tendencies rather than psychological problems.

> The company I worked for had just filed for bankruptcy, so my professional future was less than crystal clear. In addition I was tired and worn down after work, and the place wasn't heated properly. At the point where the trip peaked, it felt more intense than ever—in a negative sense. It was like I had been poisoned. Everything was just an out-of-control carousel of confusing chaos, thoughts, and physical discomfort. (Johnstad, 2021) [psilocybin mushrooms; recreational use]

Second, problem actuation must occur in synchrony with other GCMs for sustained therapeutic effects to unfold. Considering these two issues, the challenging experience construct confounds various types of experiences, each with distinct therapeutic implications: (a) experiences that are distressful but not problem-actuating, (b) experiences where a problem is indeed actuated and then changed through some corrective experience (see sections on the GCMs clarification and mastery below), (c) experiences where a problem is actuated but no corrective experience occurs, and (d) traumatically need-violating experiences that may exacerbate preexisting psychological problems or produce new ones (confirming the latter possibility, a recent study found that a small proportion of psychedelic users who had a challenging experience reported persisting functional impairment; Simonsson et al., 2023). Further complicating the interpretation of the challenging experience construct, it should not be expected that problem actuation in psychedelic states is always experienced as particularly challenging or distressful—especially not when the associated negative emotions are met with acceptance (Wolff et al., 2020). Mixed empirical results regarding the longer term effects of challenging psychedelic experiences support the view that this construct is too heterogeneous for precise conclusions about therapeutic processes to be drawn from its measurement (for a summary, see Wolff et al., 2022).

Problem Actuation and Corrective Experiences in Psychedelic States. Based on similar ideas, Roseman et al. (2019) developed the Emotional Breakthrough Inventory to measure the phenomenon of overcoming challenging emotional states during a psychedelic experience (also see our discussion under the Mastery section). Another self-report instrument, the Acceptance/Avoidance-Promoting Experiences Questionnaire (Wolff et al., 2022), was designed to capture complementary avoidance- and acceptance-related aspects of challenging psychedelic experiences on two independent main scales (each subdivided into three subscales to assess behavioral, affective, and cognitive aspects), thereby allowing for more differentiated analyses of the problem-actuating and corrective facets of challenging psychedelic experiences. In a large sample of naturalistic psychedelic users, distressful, avoidance-related experiences were negatively associated with subsequent changes in psychological flexibility—but only among participants who did not also report complementary acceptance-related experiences, which were associated with increased psychological flexibility (Wolff et al., 2022).

Shaping Favorable Conditions for Co-Occurring Problem Actuation and Corrective Experiences. The same study found that problem-oriented approach motives for psychedelic use (i.e., therapeutic intentions such as "to treat psychological problems" and "to confront difficult feelings") were positively associated with both avoidance- and acceptance-related psychedelic experiences. In contrast, avoidance-motivated psychedelic use (i.e., escapist intentions such as "to distract myself from problems" or "out of boredom") was also positively associated with avoidance-related experiences but strongly negatively associated with acceptance-related experiences (Wolff et al., 2022). A more recent survey found that "highly therapeutic experiences" (psychedelic experiences characterized by high levels of resource activation, problem actuation, and corrective experience) were associated with therapeutic use motives, whereas escapist intentions were associated with psychotherapeutically unfavorable "problem-focused experiences" (high levels of problem actuation combined with low levels of resource activation; Wolff et al., 2024). Similarly consistent with the contextual–experiential model (Figure 1), a prospective study found that ceremonial ayahuasca users who described intentions related to resolving stress associated with adverse life events more often reported re-experiencing such events later during ayahuasca ceremonies. This re-experiencing in turn predicted subsequent longer term decreases in trait neuroticism (Weiss et al., 2023). A similar longitudinal survey of naturalistic psilocybin use found that feelings of shame or guilt occurred during a majority of reported experiences, and that participants' ability to "work through" those self-conscious emotions—rather than simply their occurrence—predicted subsequent improvements in well-being (Mathai et al., 2025). While these results remain to be replicated in clinical studies, a possible implication is that the therapeutically favorable co-occurrence of problem-actuating and corrective psychedelic experiences can be purposefully promoted by therapists who use a resource-oriented, complementary approach to reduce excessive avoidance motivation and help patients build approach-motivated, therapeutic intentions prior to dosing sessions.

Therapeutic Effects Without Problem Actuation? While there is much evidence for the important role of problem actuation in psychedelic therapy, anecdotal reports question its absolute necessity. For instance, Grof et al. (1980) described treatment cases where a single extremely positive experience led to the lasting resolution of psychological problems, apparently without these problems being emotionally experienced during treatment. A recent survey study among naturalistic psychedelic users found evidence that such "resource-focused experiences" do indeed occur frequently (Wolff et al., 2024). However, whether such experiences by themselves lead to sustained symptom reductions in clinical populations remains to be investigated.

Clarification

With the GCMs mastery (discussed below) and clarification, Grawe (1997, 2004) identified two types of corrective experience that are conceptually and empirically distinguishable. Clarification (also referred to as *motivational clarification* or *clarification of meaning*) denotes the experience of gaining awareness of how one's emotional reactions relate to underlying implicit motives (i.e., hitherto unconscious wishes and fears), leading to improved self-awareness and self-understanding. The therapeutic effect of clarification experiences can be understood in terms of Lazarus's (1991) theory of emotion. Accordingly, implicit approach and avoidance goals are the relational themes that underlie the implicit appraisal of situations and events. The *primary appraisal* assesses the relevance and meaning of a situation or event with regard to one's goals. By gaining awareness of hitherto implicit avoidance goals and by relating them to negative emotional experiences, the patient is enabled to consciously reevaluate his or her negative primary appraisals.

> Do these attributions, which form the basis of the anxiety, hold up against explicit reflection and examination? Are the implicit assumptions contained in these appraisals compatible with other convictions and values of the patient? When the threatening meaning of a particular situation or event for particular goals is clarified or when the importance of these goals is revised by making explicit the implicit meanings involved, the appraisal of the situation and—at the same time—the patient's feelings may change dramatically. (Grawe, 1997)

According to Grawe (2004), clarification requires that the patient responds to the actuation of a problematic schema not with avoidance, but rather intentionally directs his or her attention to the unfolding negative emotions, sensations, thoughts, and action tendencies. Thereby, implicit appraisals can become explicitly represented in conceptual memory. Because this process requires attention to be directed against prevailing avoidance tendencies, it is more likely to succeed under favorable conditions: First, a strong intention to engage with the problem; second, a working alliance with a therapist who helps keep the patient's attention on what would otherwise be avoided.

> If both conditions are combined in therapy, there is a good chance that new contents of consciousness will emerge, representing processes that before were implicit and difficult to integrate. Because the old contents of consciousness were controlled by motivational schemata that were incompatible with the newly formed content, this process leads to the accommodation of these former schemata. Therefore, the formation of new conscious content for formerly implicit processes, which had been actively excluded from consciousness, is accompanied by a change in the structure of motivational schemata. Forming new neural connections against the inhibiting influence of already extant connections also impacts the very neural networks from which the inhibiting influence originated. This process would be called reciprocal accommodation by Piaget. (Grawe, 2004)

Clarification in Psychedelic Therapy and Psychedelic States

Since standard protocols for psychedelic therapy typically emphasize introspective attention and openness to aversive experiences (Garcia-Romeu & Richards, 2018), they appear to offer favorable conditions for clarification processes as well. Moreover, contemporary belief relaxation/modification theories suggest that psychedelics may acutely facilitate the context-dependent revision of pathological beliefs by enabling the integration of previously suppressed information (Carhart-Harris & Friston, 2019; McGovern et al., 2024; Safron et al., 2025)—a proposed mechanism that aligns with Grawe's (2004) account of the clarification process quoted above. Emerging neuroscientific evidence supports this theoretical convergence. Several studies suggest that psychedelics may impair the encoding of hippocampal-dependent episodic memories while preserving or enhancing cortical processes associated with conceptual learning (Doss, DeMarco, et al., 2024; Doss, Mallaroni, et al., 2024; Doss, Samaha, et al., 2024). This shift in memory processing may enable the updating of maladaptive emotional associations at a conceptual level, allowing previously implicit appraisals to be accessed and reorganized in more flexible, consciously accessible cognitive structures. Such a mechanism would plausibly support clarification processes as described by Grawe, in which suppressed or conflicting motivational themes are brought into awareness and integrated into a more coherent sense of self.

Clarification and Psychedelic-Occasioned Insight. Consistent with this, insights with regard to one's personality, psychological conflicts, and relationships have been recognized as a therapeutic mechanism throughout the medical history of psychedelics (e.g., Leuner, 1963). Quantitative evidence supporting the view that clarification experiences play a significant role in psychedelic therapy is provided by a recent systematic review of 98 interventional and survey studies (Kugel et al., 2025): Psychedelic-occasioned insight was positively correlated with psychedelic dose, and was rated significantly higher following psychedelics in 43 of 46 studies (93%) that included a placebo condition. Furthermore, 25 of 29 studies (86%) that analyzed associations between psychedelic-occasioned insight and longer term therapeutic outcomes found a significant association. To interpret these results, it must be noted that not all insights correspond to clarification experiences. The Altered States of Consciousness Rating Scale Insightfulness scale and the MEQ Noetic Quality scale (included in 60 and 11 studies reviewed by Kugel et al., 2025, respectively) use broadly formulated items (e.g., "I gained clarity into connections that puzzled me before"; "Gain of insightful knowledge experienced at an intuitive level") that may capture clarification-related and other psychological insights but also any non-psychological insight (e.g., about the nature of reality). The Psychological Insight Scale (Peill et al., 2022; included in five studies; e.g., "I have become more conscious of aspects of my 'self' that I used to ignore or not be fully aware of") and a single-item instrument (included in 15 studies; "How personally psychologically insightful was your experience?") focus on psychological insights but do not differentiate between clarification-related and other psychological insights, such as more resource-related insights. By contrast, the Avoidance and Maladaptive Patterns Insights subscale of the PIQ (Davis et al., 2021; included in 14 studies) focuses on clarification-related insights (e.g., "Awareness of uncomfortable or painful feelings I previously avoided"; "Realized how critical or judgmental views I hold towards myself are dysfunctional"), whereas the Goals and Adaptive Patterns subscale covers resource-related insights (see our discussion in the Resource Activation section). The PIQ has been applied primarily in retrospective survey studies, and some investigations have reported only its total score rather than the subscales. The only experimental study, a randomized controlled trial testing a dimethyltryptamine–harmine "ayahuasca analogue" in healthy subjects (Aicher, Mueller, et al., 2024), did include both subscales and found both to predict persisting positive longer term effects. To distinguish between clarification-related and resource-activating psychological insights, as well as between psychological and other insights, future clinical studies should combine global measures of insightfulness with differentiated instruments such as the PIQ or the recently developed GCMQ, which includes a Clarification scale (Wolff et al., 2024).

Autobiographical Clarification Experiences. In a clinical trial testing psilocybin for depression, most patients reported "visions of an autobiographical nature" that were regarded as insightful or informative (Carhart-Harris, Bolstridge, et al., 2018). The following report exemplifies a psilocybin-occasioned clarification experience where, after a sudden shift of perspective, the memory of a formative childhood experience appears to lose its generalized threatening meaning.

> When I was being toilet trained my mother lost it with me and drowned me in my own human waste and throughout my life, I tend to replay that. [In the dosing session], I realised that my mother was out on a ledge, we were two people out on a ledge, she too was completely unconnected, disconnected. I felt some compassion for her… a different perspective, that it wasn't an all powerful world and universe against me, my mother too was out on a ledge. (Watts et al., 2017) [depression; treated with psilocybin]

Another patient from the same study described coming to understand how internalizing his parents' emotionally invalidating attitudes as a child had engendered a persisting pattern of emotional avoidance that contributed to his problems in adulthood.

> [I] became myself at age 7, after my [grandparent] had died. I totally was back there, so vivid, so real, I had the emotions that I would have felt at the time: fearful, why did this happen, the naivety, the shock and confusion. I was getting overly upset and my parents were saying "boys don't cry." … I saw that it's not [a] weakness to be emotional, that's an unhealthy attitude. (Watts et al., 2017) [depression; treated with psilocybin]

A recurring theme in such reports is the emergence of new, less autobiographically colored appraisals that are perceived to be more "true" than previous negative appraisals.

> One participant described a very strong feeling of self-hatred, which resolved by means of a confrontation. She asked herself: "Why would you hate yourself? You're wonderful, it makes no sense … then there was an immediate relief. It was an awareness." (Belser et al., 2017) [depression/anxiety associated with a cancer diagnosis; treated with psilocybin]

> I was thinking about relationships I had with other people and thinking I could see them clearly almost as if for the first time. I had fresh insight into things. It was almost as if suddenly the scales dropped from my eyes, I could see things as they really are. (Watts et al., 2017) [depression; treated with psilocybin]

Based on the contextual–experiential model (Figure 1) and consistency-theoretical considerations, such experiences are likely facilitated by high levels of incongruence between an actuated problematic avoidance schema (including its need-violating formative biographical conditions) and a corrective, need-fulfilling immediate context in which a psychedelic experience takes place. Complementary shaping of the therapeutic relationship (Caspar et al., 2005; see our discussion under the Therapeutic Relationship section) should be considered a crucial factor in this regard.

Clarification-Oriented Interventions in Psychedelic Therapy. The contextual–experiential model (Figure 1) suggests that psychedelic-occasioned clarification experiences are most likely to occur in a psychotherapeutic context that actively pursues clarification. Although most treatment protocols used in current psychedelic therapy research involve no specific clarification-oriented interventions (but see Watts & Luoma, 2020),[3] even these protocols promote clarification, given that patients' biographies, major life events, recurrent life topics, personality, and relationship patterns are routinely discussed in preparatory sessions (albeit with the primary goal of building rapport; Garcia-Romeu & Richards, 2018). A related therapeutic consideration that highlights the importance of superordinate context factors is that psychedelic-occasioned clarification processes are not always concluded during dosing sessions but rather through subsequent integration sessions (Breeksema et al., 2020). Accordingly, clarification in psychedelic therapy is best understood not in the sense of discrete events, but rather as a continuous process that unfolds throughout preparatory, dosing, and integration sessions.

> It was different, separate fragments of that experience that all came together, colours, sounds, smells, and *afterwards when I was talking*, I started to see how they were all connected, all aspects of that experience when I was younger, it became clear that was where the problems I've had all stemmed from. (Watts et al., 2017) [depression; treated with psilocybin; our emphasis]

Clarification-Oriented Interventions in Dosing Sessions. Following current models of psychedelic therapy, verbal communication between patient and therapist during dosing sessions is kept minimal (Garcia-Romeu & Richards, 2018), leaving little room for process-directive clarification-oriented interventions. However, actively guiding patients' attention during the acute drug effect may facilitate clarification processes. Experimental evidence supporting this idea comes from a study with healthy volunteers who were prompted to recall autobiographical memories after receiving intravenous psilocybin (Carhart-Harris et al., 2012). Memory recall was rated as more vivid under psilocybin compared to placebo, and vividness ratings predicted increased well-being 2 weeks later. A corresponding clinical approach is the historical psycholytic approach to psychedelic therapy (Passie et al., 2022), where lower doses of psychedelics were used to augment talk therapy sessions embedded within longer term psychoanalytic treatment. Combining the formal framework of this historical model with the evidence-based methods of modern clarification-oriented psychotherapies (e.g., Greenberg, 2006; Sachse, 2019) is a promising avenue for future research and might in particular advance the treatment of patients who respond poorly to less process-directive interventions.

Mastery

The GCM mastery refers to a distinct type of corrective experience whose assumed therapeutic locus of action is the *secondary appraisal* (Grawe, 1997, 2004). This can also be defined as a self-efficacy expectation (Bandura, 1977) and involves an implicit assessment of how well the individual expects to be able to cope with an event or situation. According to Lazarus (1991), emotions and stress responses are essentially determined by an interplay between primary and secondary appraisals. A perceived threat to an avoidance motive (i.e., a negative primary appraisal such as "I am being treated unfairly") may elicit different emotional reactions, depending on whether the secondary appraisal is positive (e.g., "I can handle this") or negative (e.g., "This is unbearable"). In psychotherapy, a mastery-oriented approach can be considered appropriate whenever pathological significance is assigned to the secondary appraisal.

> When therapists approach their patients' problems according to the principle of mastery, they view the patients' problems in terms of being-able versus not-being-able to do something. Therapists view their patients' state or problem as concrete instances of not-being-able-to-do-otherwise, without giving the matter any deeper meaning. (Grawe, 1997)

Mastery experiences can involve different types of adaptive coping strategies (Grawe, 2004): *Problem-focused coping* (learning to resolve problem-actuating situations) is emphasized, for instance, by assertiveness or problem-solving skills training. *Emotion-focused coping* (i.e., learning to better regulate emotions associated with problem-actuating situations) is often emphasized by disorder-specific interventions such as exposure treatments, where patients confront fear-provoking situations and withhold avoidant responses. Through mastery experiences, patients learn that they can handle situations that were previously considered too difficult or anxiety-provoking to cope with. This leads to persistent reductions in avoidant coping strategies and strengthens approach-motivated mental activity (Grawe, 1997, 2004; Holtforth, 2008).

Mastery in Psychedelic Therapy and Psychedelic States

Evidence that psychedelic therapy can reduce avoidant coping comes from a recent clinical trial that compared psilocybin against

[3] Watts and Luoma's (2020) acceptance and commitment therapy-based protocol for psilocybin-augmented psychotherapy proposes a highly process-directive method, including imagination- and Focusing-based elements, to facilitate clarification experiences in preparatory and integration sessions. Barba et al. (2022) found psychological insights gained during treatment following this protocol to predict reduced depression symptoms.

the antidepressant escitalopram for depression (Carhart-Harris et al., 2021): Greater reductions in experiential avoidance were found after psilocybin, and these reductions mediated treatment outcomes (well-being, depression severity, suicidal ideation, and trait anxiety) with psilocybin, but not escitalopram (Zeifman et al., 2023). This is consistent with several qualitative studies that consistently described patient reports of reduced avoidant coping after psychedelic therapy (Agin-Liebes et al., 2024; Belser et al., 2017; Gasser et al., 2015; Watts et al., 2017).

> I can contextualize what's happening much more easily. Even if I get caught in something emotionally, I can pull back and look at the big picture, try to tone down the reactivity that may come up. I'm also very aware when I get the feeling like I can't tolerate the situation. I'm very aware that that's coming up, and what I would have done in the past was use alcohol. Now I use other tools to manage it, and the awareness that the experience will pass. (Agin-Liebes et al., 2024) [alcohol use disorder; treated with psilocybin]

In line with these observations, several experimental studies in healthy volunteers suggest that psilocybin can promote emotion-focused coping abilities related to acceptance and mindfulness (Griffiths et al., 2018; Madsen et al., 2020; Smigielski et al., 2019). Likewise, observational studies have found increases in acceptance or mindfulness capabilities following ceremonial use of ayahuasca (González et al., 2020; Mian et al., 2020; Sampedro et al., 2017; Soler et al., 2016, 2018; Uthaug et al., 2018). Besides, there is qualitative evidence from clinical trials (Agin-Liebes et al., 2024; Amada et al., 2020; Belser et al., 2017; Bogenschutz et al., 2018; Gasser et al., 2015; Lafrance et al., 2017; Malone et al., 2018; Watts et al., 2017) and quantitative evidence from studies in nonclinical samples (Domínguez-Clavé et al., 2022; Fauvel et al., 2023; Sampedro et al., 2017) that psychedelic experiences can lead to enhanced capacities for self-compassion.

Mastery Experiences in Psychedelic States. According to our extrapharmacological model (Figure 1), longer term increases in emotion regulation capacities related to mindfulness, acceptance, or self-compassion can be explained as a consequence of psychedelic-occasioned mastery experiences. The following report exemplifies a psychedelic-occasioned mastery experience that, if integrated successfully, may contribute to increased self-compassion capacities (Agin-Liebes et al., 2024; Belser et al., 2017; Bogenschutz et al., 2018; Gasser et al., 2015; Malone et al., 2018):

> I saw myself opening the fridge and wanting a beer. It was a vision, a thought, and I saw that the wanting, the needing, the craving felt like this little weasel-y animal that was like scrabbling and needing and desperate. I remember taking this little animal that I was seeing in my mind and calming it. I pet it, and I calmed it down, and I just pet this weird little alcohol weasel and was able to just calm it down and treat it lovingly. (Agin-Liebes et al., 2024) [alcohol use disorder; treated with psilocybin]

Going beyond such specific instances, the Mastery scale of the recently developed GCMQ (Wolff et al., 2024) is designed to encompass any experience associated with an increase in self-efficacy regarding one's ability to handle problematic or difficult situations (e.g., "I gained confidence in my ability to handle strong feelings"; "I found new ways to deal with situations that have caused me problems in the past"). Related instruments that may capture narrower aspects of psychedelic mastery experiences are the Acceptance-Related Experience scale of the Acceptance/Avoidance-Promoting Experiences Questionnaire (Wolff et al., 2022; $r = .79$ with GCMQ Mastery) and the Emotional Breakthrough Inventory (Roseman et al., 2019; $r = .73$; also see our discussion in the Problem Actuation section). In a clinical trial testing psilocybin-assisted psychotherapy for depression (Goodwin et al., 2025), the Emotional Breakthrough Inventory was the strongest predictor of symptom decrease following a high-dose (25 mg) psilocybin session. In another trial, psilocybin-induced emotional breakthrough also predicted reduced depression (and was itself predicted by presession therapeutic rapport; Murphy et al., 2022). In a recent naturalistic mixed-methods investigation of strategies for resolving challenging psychedelic experiences (Wood et al., 2024), emotional breakthrough was positively associated with internal (acceptance and reappraisal) and social coping strategies (disclosure and seeking support) that involve active engagement with unpleasant experiences. By contrast, supporting the view that mastery requires confrontation rather than avoidance of problem-related emotions, no such association was found with coping strategies suited to facilitate disengagement or distraction from unpleasant experiences (via sensory regulation and physical interaction).

"Learning to Let Go": Psychedelic Mastery Experiences Involving Acceptance. Consistent with this, a survey among naturalistic psychedelic users found evidence that acceptance-related psychedelic experiences (i.e., experiences of being able to accept aversive mental events, as measured by the Acceptance/Avoidance-Promoting Experiences Questionnaire) are associated with longer term increases in psychological flexibility (Wolff et al., 2022). Qualitative evidence suggests that such mastery experiences may be facilitated by acceptance-promoting learning processes within the acute psychedelic state:

> There was this huge terrifying creature with a rifle, and instead of running away, I looked at it, and it wasn't as scary as it had seemed. [My] fear subsided, it suddenly seemed ridiculous, I started laughing. If I had avoided it, it would have got more terrifying. (Watts et al., 2017) [depression; treated with psilocybin]

> The experience was of feeling desperate to flee and really then just an epiphany, I broke through and I became comfortable, my body felt so relaxed and I just fell asleep. There was an implication: I had gone through something. I had been shaking; then after I accepted it, it was like my body just relaxed. When I woke up the difference I felt wasn't gradual, it was an instantaneous huge leap towards a normal life. (Watts & Luoma, 2020) [depression; treated with psilocybin]

A common theme in these reports is that desisting from defensive resistance toward aversive contents and instead adopting an allowing, observing, or curious attitude leads to immediate relief or rewarding insights. Considering such reward contingencies from a behaviorist perspective, we have proposed that operant conditioning may, under favorable circumstances, impel processes of "learning to let go," where reinforcement of nonavoidant responses promotes experiential openness and mindful acceptance (Wolff et al., 2020, 2022). Implicit operant learning, which may be promoted by psychedelic-induced increases in reward sensitivity (Kanen et al., 2023), is presumably often accompanied by the acquisition of explicit ability knowledge:

> Now there was a familiarity to this inner space, and I found I was able to navigate much more easily than during the previous sessions. I felt no

> fear and was able to keep an even keel emotionally and psychologically, and to largely maintain the role of observer. I told [one of the supervising therapists] it was like learning to ride a bicycle and leaning into the turns. (Estevez, 2013; quoted by Letheby, 2021) [healthy volunteer; psilocybin]

Accordingly, the ability that is acquired by learning to navigate the psychedelic experience—relating to mental states and events with an open, curious, and accepting attitude—can be similar or identical to the skill that is deliberately cultivated in formal mindfulness practice (Letheby, 2021; Wolff et al., 2020). Further supporting the view that "letting go" and allowing the experience to take its course is an important aspect of psychedelic-occasioned mastery experiences, a recent naturalistic study conducted within Switzerland's limited medical use program found that ratings of relaxation during LSD and psilocybin sessions were the strongest predictor of antidepressant response (Calder et al., 2024).

Psychedelic Mastery Experiences Involving Situation-Focused Coping. Given that prevailing models of psychedelic therapy encourage introspective states during dosing sessions, it seems appropriate to first consider the potential for learning emotion-oriented coping skills such as those discussed up to here. But it is conceivable that psychedelic mastery experiences involve situation-focused coping abilities, too. Of particular interest in this regard is the phenomenon of psychedelic-induced mental "alternative simulations," that is, scenic-interactive experiences allowing experimentation with new strategies for coping with problem-actuating events (Passie, 2012; Passie & Dürst, 2008).

> There was a situation that was about the relationship between my clinging mother and me, and I said: She is always in my room, she never goes out of my life. During the session I experienced it as a child, that she is always in my childhood room. … Whatever I'm doing, she's always there; and then I really loudly shouted into the room: Go away, get out, yes, get out! [I tried] a thousand possibilities, in an almost childlike joy of experimentation: What sound will make her finally leave? Then I consciously felt: I can't do it physically. I felt small, but not hopelessly inferior, and suddenly I developed strategies for getting her out of there. … I screamed, scolded, stamped my foot … and then I suddenly developed creative ideas. … and then simply—this was a wonderfully simple solution—to say: Well, then I'll just go outside, yes. And then, within this inner image, I left; let her stay in the room, have fun!. (Passie & Dürst, 2008) [depression; treated with MDMA; our translation from German]

Although this phenomenon has been primarily described for therapy with entactogens (Feduccia & Mithoefer, 2018; Passie, 2012) and lower dose classical psychedelics (Sandison, 1954), comparable observations have also been made with higher doses of classical psychedelics (e.g., Watts et al., 2017). Acute effects of classical psychedelics that may be conducive to such experiences include intensified mental imagery (Preller & Vollenweider, 2018), altered social cognition (Preller & Vollenweider, 2019), and changes in mentalization processes (Fischman, 2022).

Mastery-Oriented Interventions in Psychedelic Therapy. Standard protocols for preparatory sessions in clinical psychedelic research commonly involve mastery-oriented interventions, including psychoeducation about attitudes and behaviors that are considered helpful for navigating challenging psychedelic experiences. Here, approach-motivated attitudes are encouraged and avoidance is discouraged. Metaphors, imagination techniques, and experiential exercises (including mindfulness- and compassion-based exercises) are used to foster the intention to allow, observe, and accept aversive experiences rather than resist them (Garcia-Romeu & Richards, 2018; Thal, Wieberneit, et al., 2022; Watts & Luoma, 2020). Based on the contextual–experiential model (Figure 1), it is assumed that these practices facilitate the occurrence of mastery experiences in subsequent dosing sessions, but this remains to be confirmed in observational and experimental studies.

Disorder-specific mastery-oriented interventions have been primarily used in studies testing psilocybin for substance use disorders (Bogenschutz et al., 2015, 2022; Johnson et al., 2014). Further disorder-specific treatments with a strong orientation toward the GCM mastery, such as psychedelic-augmented dialectical behavior therapy for borderline personality disorder (Zeifman & Wagner, 2020), have been proposed.

Facilitation of Mastery in Psychedelic Dosing Sessions. Following standard protocols used in current clinical research, patients are encouraged to engage in introspection during dosing sessions while wearing eyeshades and headphones (Garcia-Romeu & Richards, 2018). This approach naturally limits the scope for process-directive therapeutic action. Rather than exerting direct control on the therapeutic process, therapists aim to promote the spontaneous occurrence of therapeutic experiences, including mastery experiences, by creating favorable contextual conditions through careful preparation and providing a need-complementary therapeutic setting. Spontaneously occurring problem actuation may then often entail corrective experiences of mastery and clarification without direct therapeutic intervention. In this model, the opportunity—and obligation—to actively facilitate mastery experiences in a more process-directive manner arises primarily when patients are faced with challenges that overextend their coping abilities. While the literature currently lacks evidence-based guidelines to inform therapeutic action in such situations, a mastery-oriented approach would prescribe a stepwise procedure where therapists first aim to foster self-efficacy by encouraging engagement with aversive contents before, if still required, resorting to interventions that facilitate disengagement from aversive contents and assign the patient a passive role in coping with the situation (e.g., distraction, modification of the stimulus environment, or use of benzodiazepine "rescue medications"). This would be consistent with established practices for preventing premature task termination in comparable mastery-oriented psychotherapies that require extreme emotionalization of patients, such as exposure treatments.

Integration of Psychedelic-Occasioned Mastery Experiences. Some popular narratives portray psychedelics as "magic pills" that resolve problematic psychological patterns once and for all. From a consistency-theoretical perspective on psychological change, it is more realistic to assume that maladaptive response tendencies persist even after the most therapeutic psychedelic experiences. Then, successful integration requires that new, healthier response tendencies reinforced in the psychedelic state be practiced in everyday problem situations.

> I feel more positive about being able not to drink. I know there will be those times there's a craving and it's there and the opportunity to do it but then maybe having a chance to tell myself … bring out the tools from the session that I felt and see if they work, try to use those. (Nielson et al., 2018) [alcohol use disorder; treated with psilocybin]

> After doing ayahuasca, it's, it's not like I could say "Oh, because I've done ayahuasca I'm not going to diet anymore" or "Because I did

> ayahuasca I'm not going to compulsively exercise anymore." It wasn't that—it was, well, I feel more able to be with myself. I feel more capable of experiencing my emotions. So that I don't go to those behaviors that shove those emotions down that I don't want to experience anymore. (Lafrance et al., 2017) [anorexia nervosa; treated with ayahuasca]

With repeated practice, such new tendencies may develop into stable adaptive coping strategies that can reliably outcompete extant maladaptive ones. The integration process is therefore best understood as a matter of effortful volitional control, likely in many cases requiring further psychotherapeutic interventions. Therapy protocols with a strong focus on specific mastery-oriented integration methods, including mindfulness-based (Payne et al., 2021) and compassion-focused interventions (Pots & Chakhssi, 2022), have been proposed.

Implications for Research and Clinical Practice

Here, we have argued that psychedelic experiences corresponding to Grawe's five GCMs mediate psychedelic-occasioned psychological change and contribute substantially to the efficacy of psychedelic therapy. We have further suggested that GCM-related psychedelic experiences and their longer term effects are context-dependent, and that psychedelic therapy can be viewed as the systematic effort to establish favorable contextual conditions for their occurrence and beneficial integration. In summary, we propose that the therapeutic effects of psychedelic therapy (and salutogenic psychedelic use more broadly) depend at least to a substantial extent on the same psychological change processes that underlie the efficacy of all effective psychotherapies. The corresponding view that psychedelic therapy is psychotherapy (Gründer et al., 2024) concurs with the call to investigate this form of treatment with established methods of psychotherapy research (Aday et al., 2024). Consequently, we propose to systematically assess GCMs in future clinical trials. To this end, a new self-report instrument, the GCMQ (Wolff et al., 2024), was recently developed. Compared to existing GCM measures (e.g., Flückiger, Regli, et al., 2010; Mander et al., 2013), the GCMQ is less exclusively tailored to the conventional talk therapy situation and can therefore also be used to assess GCMs in the context of psychedelic dosing sessions. With this instrument and others to be developed, it will be possible to longitudinally assess GCM-related experiences as they unfold throughout the continuous psychotherapy process that spans preparatory, dosing, and integration sessions.

To date, there is no broad consensus in medicine and psychotherapy as to whether psychedelics are effective psychotherapeutic tools. A useful analogy for the potential implications of such a consensus is the now seemingly self-evident use of anesthetic drugs to facilitate surgical treatments (Stauffer, 2025; Wolff et al., 2025). The fact that acute anesthetic states facilitate surgery (and other medical treatments) is so undisputed today that approval studies for anesthetics only test whether the drug under consideration safely and effectively induces these acute states. An analogous consensus on the psychotherapeutic value of psychedelic states would presuppose that the context-dependent effects of psychedelic interventions on psychotherapeutic processes are well characterized and understood. In psychotherapy research, various experimental methods to evaluate cause–effect relationships between therapy elements, therapy processes, and therapy outcomes have been established, including dismantling, additive, parametric, and catalytic study designs (Borkovec & Sibrava, 2005). Combined with longitudinal assessment of GCMs, these approaches could be used to test the fundamental assumption that psychedelic states can be conducive to psychotherapy processes and outcomes, and thereby help establish an evidence base for psychedelics as psychotherapeutic tools. Since GCMs are assumed to underlie the efficacy of any effective psychotherapy, regardless of theoretical orientation or "therapy school," this approach could be taken with various psychotherapeutic frameworks for psychedelic therapy.

What Uses of Psychedelics Should Be Considered Psychotherapy?

Evidence for experiential mechanisms of psychedelic-occasioned psychological change comes not only from clinical studies but also from experimental studies with healthy volunteers and survey studies of naturalistic psychedelic use. As reviewed in the present article, many nonclinical studies suggest that psychotherapeutic or psychotherapy-like experiences are common even outside of the context of formal psychotherapy. Consistent with this, a recent survey found that GCM-related psychedelic experiences also occur—albeit less frequently—outside of settings designed for therapeutic purposes (Wolff et al., 2024). However, psychedelic use outside of formal clinical treatments leading to GCM-related experiences (e.g., resource-activating psychedelic experiences at music festivals, clarification experiences in ayahuasca ceremonies, mastery experiences occasioned by self-medication with psychedelic drugs) should not be confused with psychotherapy. Rather, it must be acknowledged that adaptive psychological change often occurs through diverse, sometimes spontaneous life experiences that elicit coping, reappraisal, or personal insight—capacities fundamental to human resilience and not confined to psychotherapeutic contexts (Bridges & Bridges, 2019; Lazarus & Folkman, 1984).

Psychotherapy, as distinguished from spontaneous psychological change, is characterized not merely by the occurrence of certain experiences (e.g., resource-activating, problem-actuating, mastery, or clarification experiences), but by the systematic, professional effort to establish a favorable relational context for the occurrence and integration of such experiences (Wampold & Imel, 2015). From this perspective, all psychedelic therapy models published to date—including those described as "psychological support" rather than "psychotherapy"—should be considered as forms of psychotherapy. Even minimal "psychological support" models apply process-directive psychotherapeutic interventions throughout preparatory, dosing, and integration sessions (Tai et al., 2021; Watts & Luoma, 2020). For example, as reviewed above, building the therapeutic relationship is a primary purpose of preparatory sessions, and standard preparatory interventions can be mapped onto GCMs such as resource activation (e.g., intention-setting) and mastery (e.g., encouragement to accept or "lean into" unpleasant feelings). Thus, despite differences in terminology and theoretical framing, current psychedelic therapy models consistently involve structured efforts to support the emergence and integration of psychotherapeutically relevant experiences.

At the same time, meaningful variation exists across approaches, particularly in the integration of disorder-specific interventions, the degree of process- and content-directiveness, and the explicitness that GCMs are targeted with. For example, compared to the historical "psycholytic" model, which employed intensive clarification-oriented interventions during dosing sessions, the modern standard approach to dosing sessions is considerably less process-directive.

However, consistent with findings from broader psychotherapy research that highlight the importance of secondary or emergent therapeutic processes (Grawe, 2004), clarification experiences still appear to emerge spontaneously during dosing sessions even without immediate therapist facilitation. Thus, in our view, distinctions between different approaches to psychedelic therapy are best conceptualized dimensionally, for example, by the degree of process-directiveness. By contrast, imposing categorical separations between “psychotherapy” and “psychological support” risks obscuring the shared reliance on structured relational practices that systematically create favorable contextual conditions for the emergence of common psychotherapeutic change processes.

Training Psychedelic Therapists

The existing literature on psychedelic therapist competences is thin and, echoing the culture of exceptionalism that still pervades parts of the field (Aicher, Wolff, & Herwig, 2024; Johnson, 2020), often emphasizes rather specific qualifications such as firsthand experience with altered states of consciousness, certain “spiritual” capacities, or knowledge of (alleged) traditional healing practices involving psychedelics (Gründer & Jungaberle, 2021). There are indeed some specific skills and knowledge that should be considered essential for psychedelic therapists to work safely and effectively (Wolff, Rutrecht, et al., 2025). However, if psychedelic therapy ultimately relies on the same principles of change that underlie the efficacy of all effective psychotherapies, it can be argued that psychedelic therapists must first and foremost be competent psychotherapists. Then, a crucial competency specific to psychedelic therapy is the ability to recognize, understand, and shape “ordinary” psychotherapeutic processes in the context of psychedelic experiences—without being distracted by the superficial strangeness of these experiences or the exceptionalist narratives that surround them.

Understanding psychedelic-occasioned psychological change through the lens of GCMs allows to conceptualize psychedelic treatments as one continuous, integrated psychotherapy process throughout preparatory, dosing, and integration sessions. By paying equal attention to all five GCMs rather than selectively focusing on the more narrowly defined change mechanisms that are considered important by traditional therapy schools, clinicians and researchers may more comprehensively explore, and thus more fully exploit, the psychotherapeutic potential of psychedelics.

From Confession to Profession

In the title of his most heatedly debated work, a comprehensive meta-analysis of 897 psychotherapy studies aimed at identifying GCMs and distinguishing between effective and ineffective therapeutic methods, Grawe provocatively asserted that psychotherapy must progress “from confession to profession” (Grawe et al., 1994). By this, it was meant that the field should abandon the untenable dogmas of its original traditions, overcome tendencies for therapy-school-related identity politics, and instead orient itself on the grounds of empirical psychotherapy research. In recent years, similar voices have been raised by psychedelic researchers, expressing the need to overcome the historically rooted problem of psychedelic exceptionalism (Johnson, 2020) and the related need to integrate psychedelic therapy within evidence-based psychotherapeutic treatment paradigms (Gründer & Jungaberle, 2021; Yaden et al., 2020, 2022).

With the present article, we hope to have clarified that cross-disciplinary exchange between psychedelic and psychotherapy research can make valuable contributions in these regards. Looking through the lens of Grawe’s consistency theory and GCMs, we have outlined a comprehensive theoretical understanding of psychedelic-occasioned psychological change. Central to this understanding is the view that psychedelic therapy uses the unique pharmacological properties of psychedelic substances to facilitate the same processes of change that operate in all effective psychotherapies. Besides Grawe’s model, other compatible conceptualizations of psychotherapeutic common factors (e.g., Frank & Frank, 1993; Goldfried, 1980; Wampold & Imel, 2015) may provide valuable complementary perspectives (see Greenway et al., 2020; Nayak & Johnson, 2021). Further connecting the dots between psychedelic and psychotherapy research—two historically estranged fields that hardly interact to date—is an overdue endeavor of psychotherapy integration. In the future, an important aspect of this will be the use and adaptation of established psychotherapy research methodology for clinical psychedelic research. Like other psychotherapeutic tools, psychedelics have the potential to contribute not only to successful treatment courses but also to unsuccessful and harmful ones (Bradberry et al., 2022). Curiosity, mutual understanding, and collaboration between psychedelic and psychotherapy researchers are needed to shape the development and regulation of an evidence-based, effective, and safe psychedelic-augmented psychotherapy. We are at the beginning of a fascinating time of transdisciplinary explorations.

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Received November 8, 2023
Revision received June 12, 2025
Accepted July 28, 2025 ■